

Session Objectives

At the conclusion of the session/course, the student should be able to:

1. Identify patients with anxiety by utilizing the DSM-5 criteria to differentiate between:
 - general anxiety disorder
 - panic disorder
 - social anxiety disorder
2. Recall epidemiologic information related to anxiety disorders to develop a framework for identifying those at risk of these disorders
3. Create a differential diagnosis of anxiety disorders including psychiatric and medical causes
4. Develop a treatment plan for patients diagnosed with an anxiety disorder
5. Identify risk factors that may affect outcomes in patients with anxiety disorders

Normal:

Universal human experience

Can be beneficial in many events

Pathological when the following criteria are affected:

Excessive

Intensity

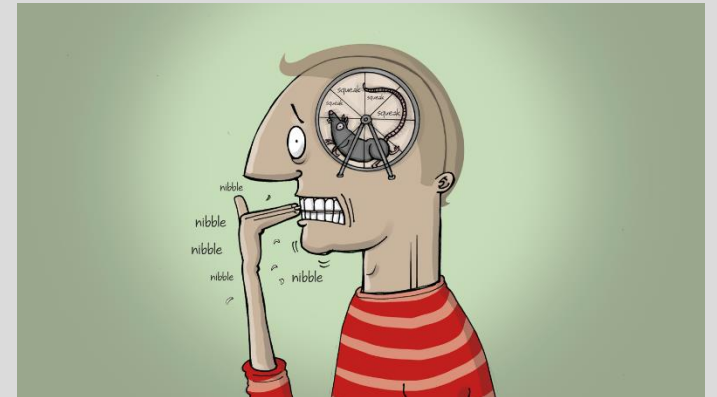
Chronicity

Impairment

Any worry or anxiety that negatively affects an individual's ability to perform daily tasks should be investigated as a possible anxiety disorder.

Any worry or anxiety that negatively affects an individual's ability to perform daily tasks should be investigated as a possible anxiety disorder.

Anxiety is manifest in physical, affective, cognitive, and behavioral domains.



Common findings among Anxiety Disorders

Source: Anxiety Disorders. Massachusetts General Hospital Comprehensive Clinical Psychiatry, Chapter 32, 353-366.2016

Physical

- Increased autonomic response

Affective/ Emotional

- Range of feelings of fear/uneasiness, edginess to terror and panic

Cognitive

- Worry, apprehension, and train of thoughts related to real or perceived threats, catastrophic thinking

Behavioral

- Responses such as avoidance, escape, and safety-seeking behaviors

Most common factors that lead to AD:

- genetics,
- brain chemistry,
- personality,
- life events,
- adverse childhood experiences (ACEs)* are stressful or traumatic events (such as physical, sexual, or emotional abuse or neglect) that occur during childhood,
- In most AD, dysregulation and low levels of γ -aminobutyric acid (GABA) and serotonin likely play a role,
- The amygdala- a portion of the middle brain that processes emotion, memory, and fear, is a key area of interest on brain-imaging studies related to anxiety disorders.

**Roughly 40% of people in the United States report experiencing two or more ACEs.*

Research on anxiety disorders in under represented groups

Source:

1. Ching, T. H. W. (2021). Culturally Attuned Behavior Therapy for Anxiety and Depression in Asian Americans: Addressing Racial Microaggressions and Deconstructing the Model Minority Myth. *Cognitive and Behavioral Practice*.
2. Laura J. Samander, & Jeffrey Harman. (2022). Disparities in Offered Anxiety Treatments Among Minorities. *Journal of Primary Care & Community Health*, 13.
3. Chodzen, G., Hidalgo, M. A., Chen, D., & Garofalo, R. (2019). Minority Stress Factors Associated With Depression and Anxiety Among Transgender and Gender-Nonconforming Youth. *Journal of Adolescent Health*, 64(4), 467–471.

DO THIS:

- Know your blind spots (your beliefs about eating disorders, substance use/dependence, etc)
- Catch your microaggressions,
- Discuss mental illness as a biological disorder rather than a personalized issue,
- Work hard to gain your patient's trust: Know that people from underrepresented groups may have experience marginalization, including in healthcare.

Generalized Anxiety Disorder (GAD)

Source: Anxiety Disorders. Massachusetts General Hospital
Comprehensive Clinical Psychiatry, Chapter 32, 353-366.2016

Symptoms associated with GAD: **WATCHERS**

- a. **W**orry,
- b. **A**nxiety,
- c. **T**ension in muscles,
- d. **C**oncentration difficulty,
- e. **H**yperarousal (or irritability),
- f. **E**nergy loss,
- g. **R**estlessness,
- h. **S**leep disturbance

Worry and tension about a variety of events and activities in one's daily life that are:

Persistent,
Excessive,
Difficult-to-control.



shutterstock.com • 370187477

Epidemiology of GAD

Female>>Male

Unmarried/minority, low SES>>

Onset at late teens to early 20s, but may occur at any age



Sex:
2:1 F:M



- Higher risk among: unmarried, minority populations, lower socioeconomic status groups
- Up to 80% have other psychiatric disorder including depression
- Also seen in patients with unexplained poly-organ symptoms

DSM-V Diagnostic Criteria for GAD

THE DIAGNOSTIC AND STATISTICAL MANUAL OF MENTAL DISORDERS , ED 5, (COPYRIGHT 2013).
AMERICAN PSYCHIATRIC ASSOCIATION.

DSM5 Diagnostic Criteria

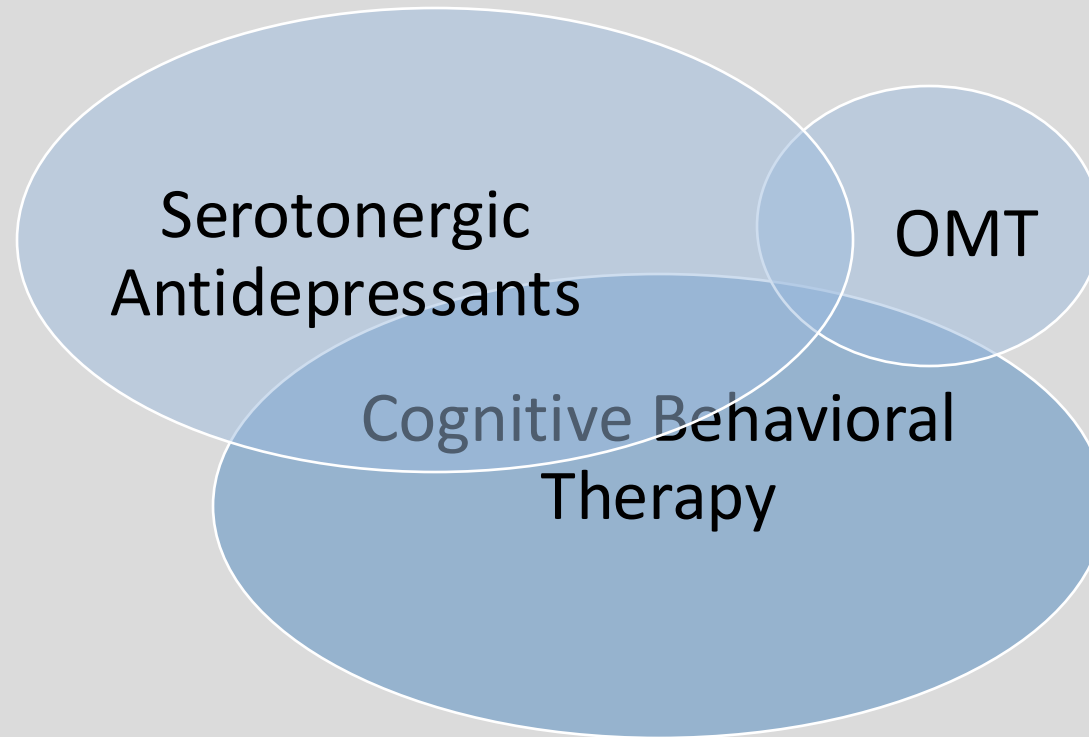
- **At least 6 months**, more days than not with:
- Associated with **3 or more** of the following:
 - Restlessness or feeling on edge
 - Being easily fatigued
 - Difficulty concentrating or mind going blank
 - Irritability
 - *Muscle tension*
 - Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep)
- **Causes distress or impairment in social, occupational or other life functions**
- **Not attributed to substance use or medical causes**
- **Is not explained by another mental disorder**

Treatment Approach to Generalized Anxiety Disorder

Hunot V, Churchill R, Silva de Lima M, Teixeira V. Psychological therapies for generalised anxiety disorder. Cochrane Database Syst Rev 2007

Academic Journal

By Reinecke, Andrea; Hoyer, Jürgen; Rinck, Mike; Becker, Eni S.. In *Journal of Behavior Therapy and Experimental Psychiatry*. March 2013 44(1):1-6.



CBT treatment for GAD

National Institute for Health and Clinical Excellence, 2007. National Institute for Health and Clinical Excellence : Nice guideline 22 – Anxiety: Management of anxiety (panic disorder, with or without agoraphobia, and generalised anxiety disorder) in adults in primary, secondary and community care.

- CBT approaches are recommended as first-line interventions in GAD
- experience of habituation to fear stimuli during repeated exposure or relaxation
- effective in reducing attentional bias to threat
- Other options (after first line options and CBT failure):
 - Benzodiazepines
 - Lorazepam
 - Sedating antidepressant
 - Mirtazapine
 - Antipsychotic medications
 - Quetiapine
- Seratogenic first line
 - SSRI or SNRI:
 - Dosing similar to depression
 - Titrate weekly or biweekly
- Partial response can be augmented with:
 - Buspirone or pregabalin
- Maintain treatment at least 12 months

**** These have risks that must be weighed before considering and should only be considered after failure of treatment options ****

Psychopharmacological approaches to treating Anxiety Disorders

Huh J, Goebert D, Takeshita J, et al. Treatment of generalized anxiety disorder: a comprehensive review of the literature for psychopharmacologic alternatives to newer antidepressants and benzodiazepines. Prim Care Companion CNS Disord 2011;13.

Option #1 for Medications for GAD

- Serotonergic first line
 - SSRI or SNRI:
 - Dosing similar to depression
 - Titrate weekly or biweekly
- Partial response can be augmented with:
 - Buspirone or pregabalin
- Maintain treatment at least 12 months

Option #2

- *Other options (after first line options and CBT failure):*
 - *Benzodiazepines*
 - *Lorazepam*
 - *Sedating antidepressant*
 - *Mirtazapine*
 - *Antipsychotic medications*
 - *Quetiapine*

*** These have risks that must be weighed before considering and should only be considered after failure of treatment options*

1. Panic Disorder

1. presence of recurrent panic attacks: abrupt, intense bursts of anxiety or fear,
2. accompanied by an array of physical symptoms (e.g., rapid heart rate, dizziness, shortness of breath, sweating, nausea).



Epidemiology of Panic Disorder



Sex:
2:1 F:M



- Onset at late adolescence to early 20s
- Often affects ability to function at work, social, and family activities
- Up to 80% have other psychiatric disorder including depression
- Poor outcomes increased in these patients



DSM-V Diagnostic Criteria

THE DIAGNOSTIC AND STATISTICAL MANUAL OF MENTAL DISORDERS , ED 5, (COPYRIGHT 2013). AMERICAN PSYCHIATRIC ASSOCIATION.

Recurrent unexpected panic attacks defined as an abrupt surge of intense fear or discomfort that reaches a peak within minutes, and during which time **four or more** of the following symptoms occur:

Palpitations, pounding heart, or accelerated heart rate

2. Sweating

3. Trembling or shaking

4. Sensations of shortness of breath or smothering

5. Feelings of choking

6. Chest pain or discomfort

7. Nausea or abdominal distress

8. Feeling dizzy, unsteady, light-headed, or faint

9. Chills or heat sensations

10. Paresthesias (numbness or tingling sensations)

11. Derealization (feelings of unreality) or depersonalization (being detached from oneself)

12. Fear of losing control or “going crazy”

13. Fear of dying.

- At least one of the attacks has been **followed by 1 month or more of one or both** of the following:

- **Persistent concern or worry about additional panic attacks** or their consequences (e.g., losing control, having a heart attack, “going crazy”).
- **A significant maladaptive change in behavior** related to the attacks (e.g., avoidance).

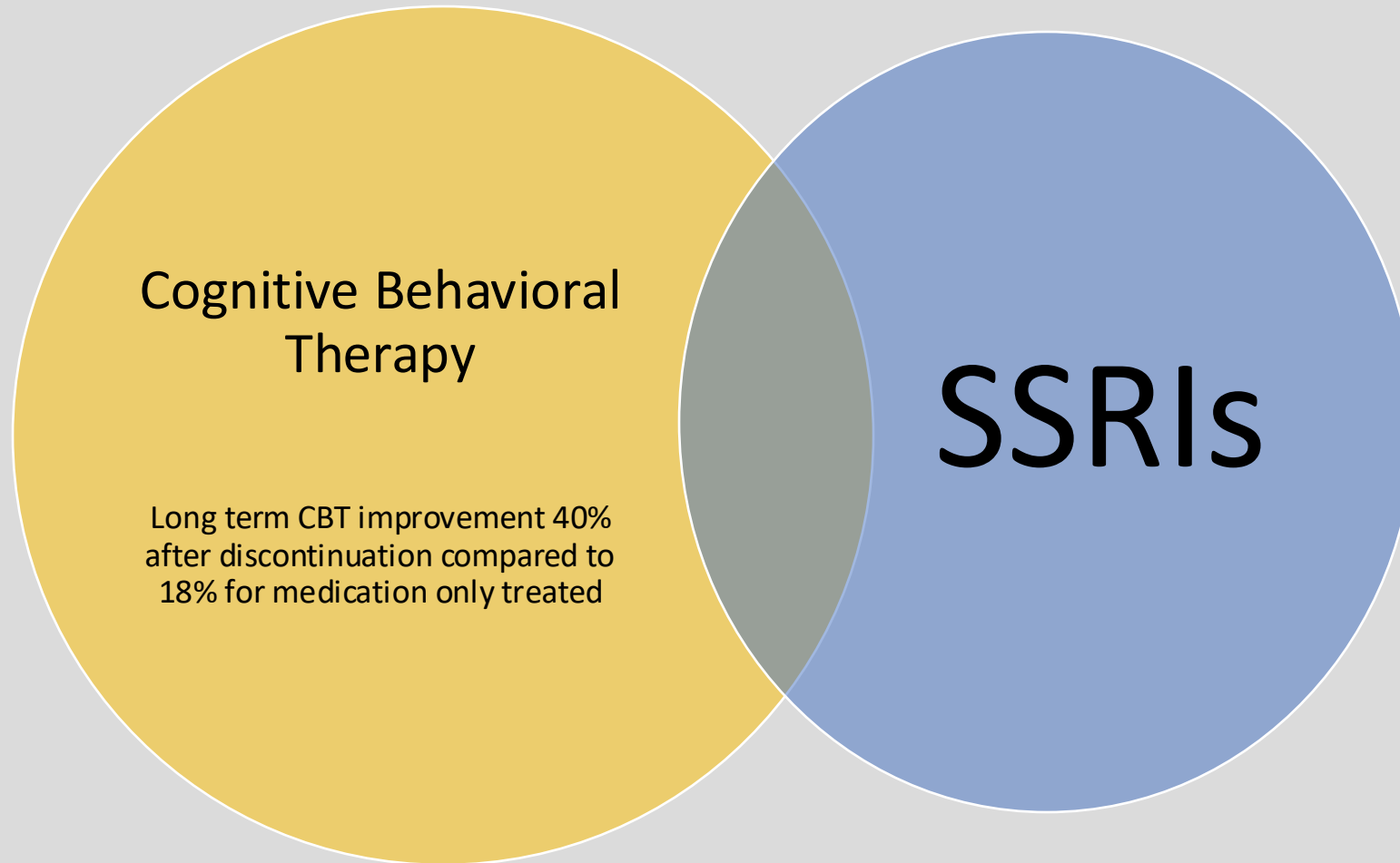
- The disturbance is **not attributable to the physiological effects of a substance or another medical condition**

- The **disturbance is not better explained by another mental disorder**

- The disturbance is not better explained by another mental disorder (e.g., the panic attacks do not occur only in response to feared social situations, as in social anxiety disorder; in response to circumscribed phobic objects or situations, as in specific phobia; in response to obsessions, as in obsessive-compulsive disorder; in response to reminders of traumatic events, as in posttraumatic stress disorder; or in response to separation from attachment figures, as in separation anxiety disorder)

Treatment for Panic Disorder

Barlow DH, Gorman JM, Shear MK, Woods SW. Cognitive-behavioral therapy, imipramine, or their combination for panic disorder: A randomized controlled trial. JAMA 2000; 283:2529.



Panic Disorder: Medication Treatment

Furukawa TA, Watanabe N, Churchill R. Combined psychotherapy plus antidepressants for panic disorder with or without agoraphobia. Cochrane Database Syst Rev 2007; :CD004364.

- First Line Psychotropic Option:
 - SSRI
 - citalopram, escitalopram, fluoxetine, paroxetine, sertraline
- Second Line Option:
 - SNRI
 - Venlafaxine (higher side effects)
 - Avoid TCAs, MAOI and benzodiazepines due to high risk of side effects and potential for abuse or overdose
- Patients with suicidality or other mental disorders should be treated with BOTH SSRIs and CBT

Epidemiology of Social Anxiety Disorder

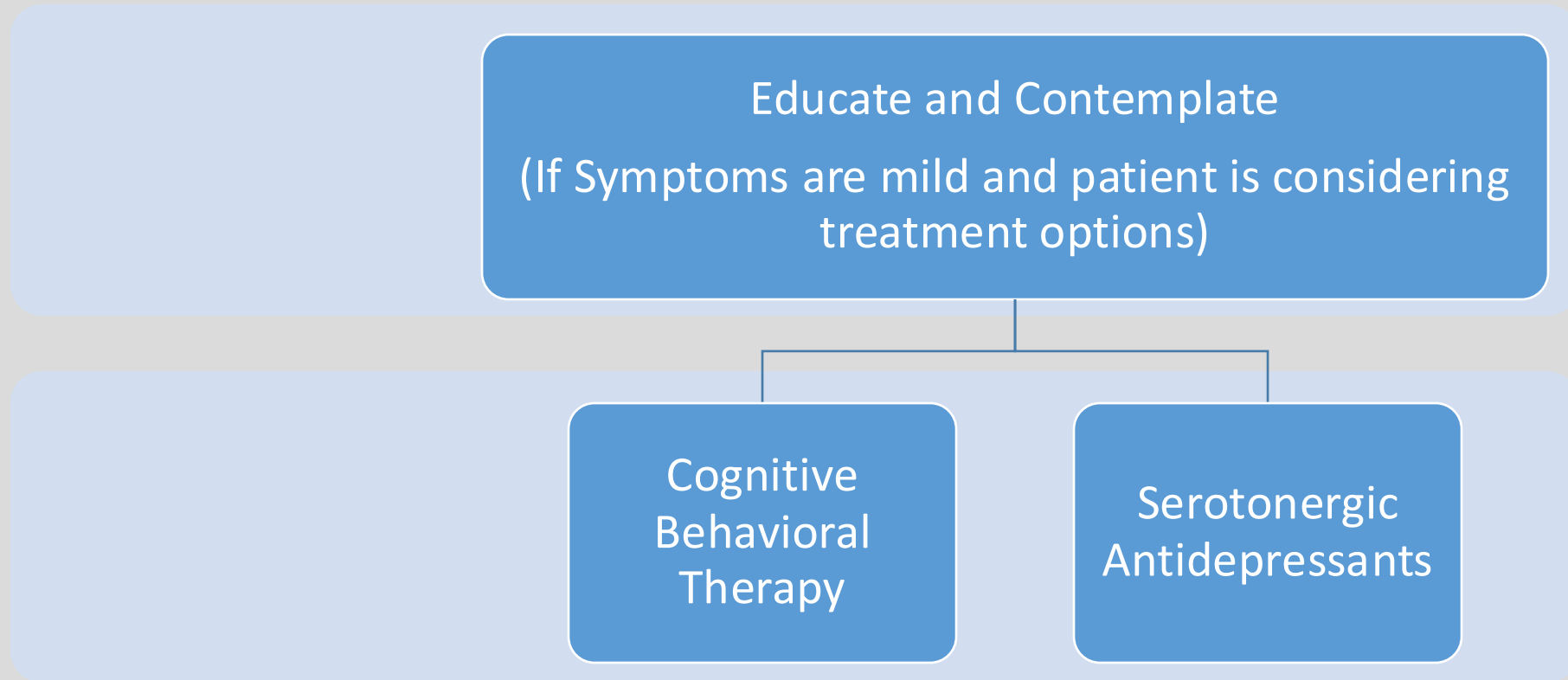


- Sex:
3:2 F:M
- *Equal number of M and F present for care*
- When affecting men has poorer outcomes for female patients
- Onset at late adolescence to early 20s
- Often affects ability to function at work, social, and family activities
- Up to 80% have other psychiatric disorder including depression
- Poor outcomes increased in these patients

Treatment for Social Anxiety Disorder

Stein MB, Stein DJ. Social anxiety disorder. Lancet 2008; 371:1115.

Blanco C, Heimberg RG, Schneier FR, et al. A placebo-controlled trial of phenelzine, cognitive behavioral group therapy, and their combination for social anxiety disorder. Arch Gen Psychiatry 2010; 67:286.



Medications for Social Anxiety Disorder

Pollack MH, Van Ameringen M, Simon NM, et al. A double-blind randomized controlled trial of augmentation and switch strategies for refractory social anxiety disorder. Am J Psychiatry 2014; 171:44.

- Seratogenic first line:
 - SSRI or SNRI
 - Dosing similar to depression
 - Titrate weekly or biweekly
- If treatment not effective within 10-12 weeks then change medication or include CBT
- Avoid benzodiazepines due to high risk of substance abuse

Medication for Social Anxiety Disorder, Performance Only

Steenen SA, van Wijk AJ, van der Heijden GJ, et al. Propranolol for the treatment of anxiety disorders: Systematic review and meta-analysis. J Psychopharmacol 2016; 30:128

- First Line:
 - **CBT followed by exposure therapy**
- If single or rare performance activities:
 - **Beta-blocker**
 - Propranolol
 - Benzodiazepine
 - Low dose clonazepam with caution

